

Prediction of Chronic Disease Risk Using Nutrition, Lifestyle & Clinical Data from The Irish Longitudinal Study on Ageing (TILDA)



Abderahman Haouit

CSIS Research Centre

Department of Computer Science and Information Systems

Faculty of Science and Engineering

University of Limerick

Submitted to the University of Limerick for the degree of

Artificial Intelligence (MSc) 2026

-
1. Supervisor: Dr. Alison O'Connor
Computer Science & Information Systems
University *of* Limerick
Ireland

Contents

List of Tables	iii
List of Figures	iv
List of Abbreviations	vi
1 Introduction	1
1.1 Research Problem and Motivation	3
1.2 Research Aims and Objectives	3
1.3 Contributions of the Research	4
1.4 Research Plan	5
1.4.1 Timeline and Milestones	5
1.5 Thesis Outline	5
2 Literature Review	7
2.1 Frailty and Physical Function	7
2.1.1 Probabilistic Interpretation of Frailty	7
2.1.2 Bayesian Updating of Frailty Risk	8
2.2 Mental Health in Older Adults	9
2.3 Nutrition and Biological Ageing	10
2.3.1 Bayesian Perspective on Nutritional Biomarkers	10
2.3.2 Contextual and Integrative Considerations	11
2.4 Machine Learning in Ageing and Chronic Disease	11
2.4.1 Feature Sets in ML Studies	12
2.4.2 ML Models Used	12
2.4.3 Performance and Validation	12

CONTENTS

2.4.4 Explainability in ML for Ageing	13
2.5 Gaps in the Literature	15
2.6 Chapter Summary	16
References	18

List of Tables

2.1	Summary of selected studies on ageing using TILDA	14
-----	---	----

List of Figures

- 1.1 Project timeline with tasks and milestones (June 2025 – May 2026). 5

List of Abbreviations

AI	Artificial Intelligence
AUC	Area Under the ROC Curve
CES-D	Center for Epidemiologic Studies Depression Scale
DNN	Deep Neural Network
ELSA	The English Longitudinal Study of Aging
EU	European Union
GBM	Gradient Boosting Machine
GDPR	General Data Protection Regulation
HADS	Hospital Anxiety and Depression Scale
HRS	The Health and Retirement Study, (a longitudinal study of ageing in the United States)
IDS	The Intellectual Disability Supplement
ISSDA	The Irish Social Science Data Archive
LDA	Linear Discriminant Analysis
ML	Machine Learning
RF	Random Forest
SHAP	Shapley Additive Explanations
SHARE	The Survey of Health, Aging and Retirement in Europe
TILDA	The Irish Longitudinal Study on Ageing

List of Abbreviations

WHO World Health Organization

Chapter 1

Introduction

Chronic conditions including cardiovascular disease, diabetes, and frailty are leading contributors to morbidity and mortality in ageing populations [1, 2]. These conditions are associated with declines in functional capacity (e.g., gait speed, grip strength [3]), increased healthcare utilisation [4], and reduced quality of life [5].

The financial and societal burden is substantial. In Europe, poor diet is a major risk factor for non communicable diseases, contributing billions in healthcare costs annually [6]. In Ireland, malnutrition alone is estimated to cost over €1.4 billion per year [7]. Recognising this, international and national policies including the World Health Organization (WHO) Global Action Plan on Noncommunicable Diseases and Ireland’s *Healthy Ireland* framework highlights nutrition and lifestyle interventions as central to healthcare strategy.

In parallel, (Artificial Intelligence (AI)) and (Machine Learning (ML)) are increasingly applied in healthcare to support risk prediction and personalised intervention. However, European regulations such as the General Data Protection Regulation (GDPR) and the forthcoming European Union (EU) AI Act impose strict requirements for transparency, explainability, and governance. This regulatory context underscores the importance of interpretable and ethically robust ML approaches.

Traditional risk prediction models have largely relied on clinical biomarkers such as blood pressure and lipid profiles. Yet, growing evidence shows that broader predictors including nutrition [8], physical activity [9], mental health [10],

1. INTRODUCTION

and social determinants [11] improve predictive performance. Estimating the likelihood of developing a condition given certain risk factors, help quantify the influence of these predictors in clinical decision making.

(The Irish Longitudinal Study on Ageing (TILDA))¹ provides a nationally representative cohort of adults aged 50+ in Ireland, surveyed across multiple domains in repeated waves (waves 1-5). The raw wave datasets capture the full richness of TILDA specific measures relevant to Irish health, policy, and society. In parallel, the Harmonized TILDA dataset restructures selected variables to align with the RAND (The Health and Retirement Study, (a longitudinal study of ageing in the United States) (HRS)) framework, enabling cross national comparison with equivalent studies such as HRS (US), The English Longitudinal Study of Aging (ELSA) (UK), The Survey of Health, Aging and Retirement in Europe (SHARE) (continental Europe).

This harmonisation enhances international comparability, though at the cost of excluding some study specific variables. An additional dataset, the The Intellectual Disability Supplement (IDS), extends coverage to individuals with disabilities, improving inclusivity and reducing sampling bias. Together, these resources offer both detailed national data and a platform for internationally comparable, generalisable analyses.

Despite these advantages, existing ML research on chronic disease prediction faces persistent limitations: over reliance on clinical predictors, limited incorporation of lifestyle and social variables, predominantly cross sectional study designs that fail to capture trajectories, and a lack of interpretability consistent with emerging AI governance standards.

This thesis addresses these gaps by integrating TILDA’s longitudinal, multidomain data with interpretable ML methods, demonstrating how nutrition, physical activity, and mental health can be combined with traditional clinical markers to improve prediction of ageing related outcomes.

¹TILDA data are pseudonymised and accessed under licence via The Irish Social Science Data Archive (ISSDA). Use adheres to UL S&E ethics approval and data governance requirements.

1.1 Research Problem and Motivation

Despite increasing policy interest in lifestyle and nutrition and advances in ML methods, approaches to chronic disease prediction in older adults remain limited in several ways.

- **Limited Integration of Multidimensional Data:** Most models prioritise clinical variables while underutilising modifiable lifestyle and nutrition factors [8, 12].
- **Underuse of Longitudinal Dynamics:** Few studies leverage repeated measures to capture developments of risk, such as frailty progression [13]. Probabilistic reasoning can help estimate the likelihood of developing a condition at a future time given earlier measurements, for example calculating $P(\text{Frailty at wave 5} \mid \text{Gait speed at wave 1, Nutrition at wave 2})$.

This approach helps quantify how early indicators influence later outcomes and supports better feature selection and timing of interventions in predictive models.

- **Lack of Explainability:** Many ML models are not interpretable, limiting clinical adoption and failing to meet emerging regulatory requirements for trustworthy AI in healthcare [14].

Addressing these challenges is essential to improve predictive accuracy, support evidence based interventions, and ensure alignment with evolving policy and regulatory priorities.

1.2 Research Aims and Objectives

The goal of this thesis is to develop and validate ML models that integrate nutrition, lifestyle, and mental health features with clinical data to improve prediction of chronic disease and ageing related outcomes in a representative cohort of older adults. This goal will be achieved through the following objectives:

1. INTRODUCTION

- Review existing applications of ML for chronic disease prediction in ageing populations, with emphasis on gaps in lifestyle and nutrition integration.
- Preprocess and harmonise TILDA waves 1-5, and engineer multidomain features (nutrition, activity, mobility, mental health, clinical indicators).
- Develop and compare baseline statistical models (e.g., logistic regression) with advanced ML methods (Random Forest (RF), Gradient Boosting Machine (GBM)).
- Evaluate models using discrimination (Area Under the ROC Curve (AUC)), calibration, and error metrics (accuracy, precision, recall, F1-score), perform ablation analyses to assess the incremental value of lifestyle and nutrition predictors.
- Identify actionable predictors (e.g., vitamin D status, gait speed reserve) and highlight modifiable risk factors for intervention.
- Document ethical, governance, and reproducibility practices for handling longitudinal pseudonymised health data.

1.3 Contributions of the Research

This research makes contributions at three levels:

- **Empirical:** Demonstrates how nutrition, lifestyle, and mental health features enhance prediction when combined with established clinical baselines.
- **Methodological:** Provides a reproducible pipeline for harmonising longitudinal TILDA data and integrating multidomain features into ML workflows.
- **Clinical:** Identifies modifiable predictors with relevance for targeted interventions and policy to promote healthy ageing.

1.4 Research Plan

This project is structured into sequential phases, each linked to specific milestones. The plan spans twelve months (June 2025 - May 2026) and covers literature review, ethics and data access, preprocessing and feature engineering, model development and evaluation, and thesis writing and submission.

1.4.1 Timeline and Milestones

Figure 1.1 presents the Gantt chart for the project, highlighting major tasks and deliverables. Key milestones include finalising research questions and objectives (M1), completing ethics and ISSDA applications (M2), confirming dataset access (M3), completing preprocessing and feature engineering (M4), finishing model development and evaluation (M5 - M6), drafting the full thesis (M7), and final submission (M8).

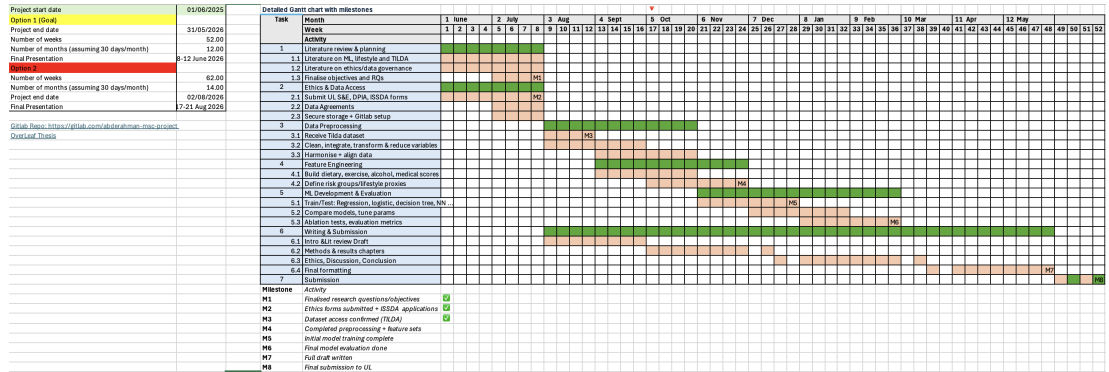


Figure 1.1: Project timeline with tasks and milestones (June 2025 – May 2026).

1.5 Thesis Outline

In *Chapter 2* a literature review on ML prediction of chronic disease in ageing populations, with emphasis on lifestyle and nutrition predictors and methodological considerations is presented. *Chapter 3* discusses the TILDA study design, data preprocessing, harmonisation, and feature engineering (Flow Charts...). In *Chapter 4* the modelling framework is outlined. This includes baseline and advanced ML, hyperparameter tuning, validation, and evaluation metrics. *Chapter*

1. INTRODUCTION

5 shows the model result, including model comparisons, assessments of feature importance, and an ablation analyses. In *Chapter 6* the implications, limitations, ethical considerations are discussed, and future directions for the research proposed. Additional supporting documents are included in appendices.

Chapter 2

Literature Review

This chapter reviews literature on ageing, chronic disease, and predictive modelling using TILDA. The focus is on three domains central to later life health: frailty and physical function, mental health, and nutrition and biological ageing. Chronic conditions including multimorbidity, frailty, and mental health difficulties are leading causes of morbidity and reduced quality of life in older adults, often overlapping to accelerate functional decline. In line with the WHO healthy ageing framework, research using TILDA highlights frailty, mental health, and nutrition as interconnected determinants of later life outcomes. The following subsections examine each domain in turn.

2.1 Frailty and Physical Function

Frailty is a reflection of higher vulnerability and declining physiological reserve. In TILDA, it is captured through both the frailty phenotype and the frailty index, with gait speed, grip strength, and mobility decline serving as practical predictors. These measures strongly predict hospitalisation and mortality.

2.1.1 Probabilistic Interpretation of Frailty

One can argue that from a probabilistic perspective, the chance of an adverse outcome (such as mortality, Y) given observed features (such as frailty markers, X) as a conditional probability $P(Y | X)$. This allows us to deduce which frailty

2. LITERATURE REVIEW

features are most predictive of adverse outcomes. Features may be independent, dependent, or mutually exclusive.

For instance, presence or absence of metabolic syndrome represents a mutually exclusive event that can inform risk models.

$$P(X_i \cap X_j) = P(X_i)P(X_j)$$

for independent features X_i and X_j , guiding feature selection and reducing redundancy in model inputs. This reflects the likelihood that an individual with certain frailty characteristics will experience a particular outcome.

Evidence suggests that frailty is dynamic, not irreversible. For example, multi state modelling showed that many prefrail adults transition back to non frail, although established frailty strongly predicts mortality [15]. Metabolic syndrome, on the other hand, speeds up the onset of frailty, which suggests a metabolic pathway of vulnerability. [16]. The presence of metabolic syndrome modifies the conditional probability $P(\text{Frailty} \mid \text{Metabolic Syndrome})$ and may increase the likelihood of adverse outcomes over time. Another detail is that age matters: obesity predicts mobility decline earlier in life, whereas later in life, traits that protect against this, including grip strength, become more significant [17].

These findings partly conflict: frailty appears both reversible and progressive, depending on risk context. This complexity highlights the need for probabilistic models that can update beliefs about risk as new features are observed, as formalised by Bayes' theorem:

$$P(Y \mid X) = \frac{P(X \mid Y)P(Y)}{P(X)}$$

where $P(Y \mid X)$ is the posterior probability of outcome Y given features X .

2.1.2 Bayesian Updating of Frailty Risk

This enables us to incorporate prior knowledge and observed data, improving the interpretability of feature outcome relationships in ageing. Here, $P(Y \mid X)$ is the posterior probability = (Likelihood * Prior / Evidence), representing the updated belief about the outcome after observing features. $P(Y)$ is the prior probability, reflecting baseline risk. $P(X \mid Y)$ is the likelihood, quantifying the

probability of observed features given the outcome, and $P(X)$ is the evidence (the overall probability of the features). This framework allows us to update predictions as new data is observed, improving interpretability in feature outcome relationships.

Limitations remain. Causal inference is limited by observational designs, and predictors are frequently examined separately, ignoring interactions. Most frailty models have only been with the TILDA dataset and with this present study focuses on the same TILDA but with an expansion on the harmonised set to explore more longitudinal prediction within this context. These gaps point to the need for multidimensional approaches. Traditional regression models may not fully capture age dependent interactions, suggesting a role for more flexible methods such as machine learning techniques and models.

2.2 Mental Health in Older Adults

Mental health, including depression, anxiety, loneliness, and social isolation, strongly shapes quality of life. Evidence from TILDA shows strong links to both physical and social factors. [18] found that participation in physical, social, and cognitive activities summarised by the Act-Belong-Commit (ABC) indicator was associated with lower depression and anxiety. This suggests social integration and lifestyle protect mental wellbeing. However, [19] reported that self reported measures may exaggerate such associations..., raising concerns of bias. These two findings conflict: the effect of social engagement is strong when self reported, weaker when objectively assessed.

Objective data strengthen the case for activity. Accelerometer based measures show a dose response link between moderate to vigorous activity and lower depressive symptoms [9]. Yet reverse pathways are also evident: incident falls and functional decline predict later depression, refereed by Fear of Falling (FOF) [20]. Together, these results reveal a bidirectional relationship, mental health both influences and is influenced by physical decline. Most studies rely on broad self report scales such as Center for Epidemiologic Studies Depression Scale (CES-D) or Hospital Anxiety and Depression Scale (HADS), with little biomarker or clinical validation.

2. LITERATURE REVIEW

This literature suggests mental health in ageing cannot be modelled as a one way effect. It requires longitudinal, multidimensional approaches that can capture mutual pathways, an area where machine learning is well suited especially when combined with probabilistic models that estimate the likelihood and posterior probability of mental health outcomes given multiple interacting features.

For example, we can estimate:

$$P(\text{Depression} \mid \text{Activity, Falls, Social Engagement}),$$

quantifying how these interacting features together influence mental health outcomes. Probabilistic modelling helps determine which features have the strongest effect and which may be redundant for predictive purposes.

2.3 Nutrition and Biological Ageing

Nutrition is increasingly recognised as a modifiable factor in ageing and frailty. Biomarkers like vitamin D, lutein, and zeaxanthin have been linked to physical and cognitive outcomes in TILDA. [8] showed that higher plasma lutein and zeaxanthin dietary carotenoids found in leafy vegetables were linked to slower frailty progression. By contrast, [21] found vitamin D deficiency predicted diabetes and musculoskeletal decline, especially among adults with low sunlight exposure. These studies point to different nutritional mechanisms: antioxidant protection versus metabolic regulation.

2.3.1 Bayesian Perspective on Nutritional Biomarkers

The effect of nutrition on ageing outcomes can be formalised probabilistically using Bayes theorem:

$$P(\text{Outcome} \mid \text{Nutritional Status}) = \frac{P(\text{Nutritional Status} \mid \text{Outcome})P(\text{Outcome})}{P(\text{Nutritional Status})},$$

where the likelihood function $P(\text{Nutritional Status} \mid \text{Outcome})$ quantifies how observed nutritional biomarkers relate to health states, and the posterior $P(\text{Outcome} \mid \text{Nutritional Status})$ captures updated beliefs about outcomes, including uncertainty in small sample studies. This helps identify which biomarkers are most

informative for predicting ageing outcomes which supports feature selection in ML models.

2.3.2 Contextual and Integrative Considerations

Malnutrition also reflects social and functional risks. [2] found that hospitalisation, functional loss, and poor social support increased malnutrition risk, with notable sex differences. This highlights a limitation in much nutritional research: biological markers are studied without integrating social context. The difference between biological and chronological age is a major point of debate. Biomarker based metrics that better capture these cumulative exposures, like frailty indices or epigenetic clocks, frequently vary from chronological age. McCarthy and Azizi link metabolic syndrome and nutritional deficits to accelerated biological ageing [22, 23]. Although the majority of the evidence is cross sectional and has little ability to capture changes over time, these indicators offer a more precise prediction of vulnerability. Reliability is also decreased in biomarker research with small sample sizes, which can be formalised through the likelihood function $P(\text{Data} \mid \theta)$ and its impact on posterior uncertainty in Bayesian inference.

Taken together, nutrition interacts with biological, metabolic, and social systems. Isolating single nutrients risks oversimplification. Integrative modelling that combines biomarkers, clinical factors, and social determinants supported by machine learning pipelines offers a more promising route. Models that estimate the joint distribution of features and outcomes can better capture the complex dependencies relevant to biological ageing.

2.4 Machine Learning in Ageing and Chronic Disease

ML is increasingly applied in ageing research because it can capture non linear and multidimensional relationships that traditional regression models, such as logistic regression, often miss [24, 25].

2. LITERATURE REVIEW

2.4.1 Feature Sets in ML Studies

Studies using ML in ageing research include many types of predictors: clinical measures, sociodemographic factors, lifestyle habits, cognitive tests, and psychosocial variables. A recurring pattern, however, is that models often underline easily measured physical health data, such as comorbidity counts, mobility scores, or vital signs, while more complex but important predictors, including nutrition and social factors, are underrepresented [26]. This imbalance may limit the ability of models to capture the full complexity of ageing, especially considering that mental health and nutrition exert independent and interacting effects on later life outcomes see (here). Models that focus only on readily available clinical data risk reproducing the same biases seen in traditional regression approaches.

2.4.2 ML Models Used

The methodological approaches used in ageing research span both traditional and advanced models. Logistic regression and Markov models remain common baselines due to their interpretability and simplicity, yet they are often unable to capture non linearities or higher order interactions. More recent work has shifted towards ensemble methods such as random forests (RF) and gradient boosting machines (GBM), as well as deep neural networks (Deep Neural Network (DNN)), which generally achieve higher predictive accuracy.

Similarly, [27] evaluated mortality prediction using a standard logistic regression model with demographic, clinical, and functional predictors, achieving an AUC of 0.78, whereas, ensemble methods in [24] reached 0.854 for diabetes prediction, showing how flexible, non linear models can better capture complex interactions among ageing related predictors.

2.4.3 Performance and Validation

Predictive performance across studies is generally moderate to strong, with AUC values between 0.70 and 0.90 for outcomes like diabetes, gait speed, depression, and mortality. This indicates that models can distinguish reasonably well between individuals who will or will not experience an outcome.

However, there are important limitations. Many studies rely only on internal validation, testing models on the same data used for training. This can inflate performance estimates and reduce confidence in applying the model to new populations. To add, Most studies are cross sectional, so causal or temporal effects cannot be assessed. When combined with the underuse of nutrition and social predictors, these issues indicate that current models may not fully represent the multidimensional nature of ageing and could produce context specific results rather than generalisable predictions [26].

2.4.4 Explainability in ML for Ageing

As ML models become more complex, interpretability emerges as a critical challenge. Explainability tools like Shapley Additive Explanations (SHAP) can highlight feature importance, but their use is inconsistent, particularly in deep learning models [3, 24, 28]. These methods improve transparency and can highlight unexpected drivers of outcomes, such as psychosocial variables being stronger predictors than biomedical ones, yet black box concerns remain, especially for deep learning models, where feature interactions are difficult to track. For ageing research, where clinical adoption requires both accuracy and trust, explainability is not optional but essential. Current efforts show promise but remain fragmented, with no consensus on best practice. This gap links directly to the broader methodological limits discussed in the next section on gaps in the literature.

2. LITERATURE REVIEW

Table 2.1: Summary of selected studies on ageing using TILDA

Theme	Author/year	Dataset	Variables/features	Methods/model type	Key Findings/outcome	Limitations
Frailty	Romero-Ortuno (2021) [15]	Wave 1-5	Frailty state, Frailty phenotype (FP) components (Exhaustion(EX), Unexplained weight loss(WL), Weakness(WK), Slowness(SL), Low physical activity(LA))	Multi state Markov model, longitudinal 8 year follow up	32% of pre frail to non frail; frailty strongly predicted 31% mortality	Self report bias; observational design
Falls + ML	Donoghue (2023) [14]	Wave 1-3	Gait, mobility, medications, history of falls	Conditional inference forest; 5-fold Cross-Validation (CV); undersampling for imbalance	Accuracy up to 69.7%; for adults aged 50-64.	Area Under the Receiver Operating Characteristic Curve (AUROC) values were ≤ 0.69 and Precision-Recall Area Under the Curve (PR-AUC) ≤ 0.39 indicating low predictive accuracy; no external validation; cross sectional; Class imbalance
FOF	Zarudsky (2014) [26]	Wave 1	Age, sex, fall history	Cross sectional prevalence analysis	23.3% prevalence; higher in women; 70% with FOF had no recent falls in the last 12 months	Self reported; cross sectional
Functional Mobility	Huang (2019) [17]	Wave 1-2	Measured by Timed Up and Go (mobility test) (TUG), grip strength, physical activity, vision, Body Mass Index (BMI), age	Linear regression, stratified analyses	Age modifies effects; Physical Activity (PA) and grip protect mobility; vision benefit only in middle aged (Beta = -0.032, P = 0.002)	Observational; 2-year follow-up
Depression	Jacob (2023) [20]	Wave 1-2	Falls, FOF, HADS-A, CES-D	Prospective logistic regression	Falls predicted incident anxiety (Odds Ratio (OR)=1.58) and depression (OR=1.43)	Self-reported falls; short follow up; mediation effect partially explored
	Laird (2023) [12]	Wave 1-5	Moderate-to-Vigorous Physical Activity (MVPA), walking, sociodemographics	Longitudinal regression; mixed model	Higher physical activity reduced depressive symptoms; dose response effect	Self-reported PA; observational; limited causal inference
Nutrition	Murphy (2023) [8]	Wave 1-5	Plasma lutein, zeaxanthin	Linear/ordinal logistic regression	Higher carotenoids linked to slower frailty progression Lower odds of becoming frail (ORs = 0.57-0.89)	Observational; not predictive of longitudinal muscle changes
	Bardon (2020) [2]	Wave 1-2	Hospitalisation, functional status, social support, sex, falls	Logistic regression; sex-stratified analyses	Functional decline, hospitalisation, poor support increased malnutrition risk; sex differences	Observational(short follow up (2 years)); limited generalisability ;65 years, self reported
ML	Xu (2023) [24]	Wave 1-3	105 predictors from 40,000+ variables (social, clinical, mental, family)	Stacked ensemble: RF, XRT, GLM, GBM, DNN; SHAP explainability	AUC=0.854 for 2 year diabetes prediction; top predictors LDL, cirrhosis, sex	No external validation; short follow up; ensemble complexity limits deployment
	Davis (2021) [3]	Wave 3	Age, grip strength, chair stands, BMI, cognitive tests	Histogram gradient boosting regression; SHAP	r^2 test: Usual Gait Speed (UGS) ≈ 0.41 , Maximum Gait Speed (MGS) ≈ 0.46 , GSR ≈ 0.21	Modest predictive power; cross sectional; no external validation
	Zhang (2025) [29]	Longitudinal IMAGEN cohort	Psychopathology, personality, cognition, substance use, environment	Regularized logistic regression; diagnostic	Longitudinal AUC: ED=0.71, MDD=0.64, AUD=0.67; shared transdiagnostic predictors	Limited age range; no external replication
	Boyle (2021) [25]	Wave 3	Brain predicted age difference (brainPAD); processing speed; attention; cognitive flexibility; verbal fluency	Linear regression, correlation analyses	Higher brainPAD associated with lower cognitive performance across multiple domains	Cross sectional; no causal inference; limited sample

2.5 Gaps in the Literature

First, most studies rely on TILDA or similar cohorts of Irish and predominantly older adults (50+). This limits confidence in how well results transfer to other populations. The present project focuses on TILDA to study ageing in depth but also makes use of the harmonised dataset, which links TILDA with comparable UK and US cohorts. This helps place findings in a broader international context while retaining a core emphasis on older adults.

Second, there is an over reliance on physical health predictors such as comorbidity counts and mobility measures, while nutrition, social, religious or spiritual, and environmental variables are often overlooked. As shown in [8], nutritional biomarkers like lutein and zeaxanthin are strong predictors of frailty, yet these are rarely integrated into broader ML pipelines. By contrast, studies such as [2] highlight the role of social support and functional decline in malnutrition risk. Together, these examples show that excluding nutrition and social factors risks producing models that reinforce the same biomedical biases seen in regression heavy work. From a Bayesian viewpoint, the omission of key features reduces the explanatory power of the model and weakens the posterior probability $P(\text{Outcome} \mid \text{All Features})$ compared to $P(\text{Outcome} \mid \text{Limited Features})$. Including a richer set of features increases the likelihood that the model accurately reflects the true data generating process.

Third, methodological choices remain conservative. Regression models still dominate, while more advanced approaches such as deep neural networks or transformers are rarely explored. For instance, [27] achieved an AUC of 0.78 using logistic regression for mortality prediction in TILDA, whereas [24] reported an AUC of 0.854 with ensemble methods for diabetes incidence. This contrast demonstrates both the dominance of regression and its weaker performance compared to flexible ML models. Where ensemble or brain age approaches have been attempted, validation is almost always internal, with little cross cohort testing. Weak validation and limited feature sets reduce the generalisability of the estimated conditional probabilities and increase the risk of overfitting. Maximum likelihood estimation and Bayesian inference can provide a formal framework for

2. LITERATURE REVIEW

assessing model fit and updating predictions as more diverse and representative data become available.

Fourth, many outcomes remain underexplored. Studies on dementia or frailty frequently focus on cognitive or physical decline while leaving aside outcomes critical to older adults, such as pain, sleep quality, and subjective quality of life. For example, [8] used cross sectional plasma biomarkers to study frailty but did not assess sleep or pain, while gait speed models such as [3] similarly neglect subjective quality of life. This shows how reliance on narrow outcomes leaves broader aspects of wellbeing unmeasured.

Fifth, cross sectional bias and data handling weaken reliability. Much nutritional work, such as [8], relies on single time point biomarker data, which cannot capture longitudinal change. By contrast, studies applying multi wave TILDA data [15] demonstrate the added value of modelling temporal dynamics. Similarly, missing data are often handled through list wise deletion, as in [27], which risks biasing results toward healthier participants, whereas more robust imputation approaches remain underused. Missing or poorly handled data can distort the likelihood function and hence the posterior predictions, reducing confidence in the model’s outputs.

Finally, interpretability challenges persist. While tools like SHAP have been applied to highlight feature importance [24, 28], their use is inconsistent, and consensus on best practice has yet to emerge. Without transparent explanations, even accurate models may face barriers to clinical acceptance. Taken together, these examples show that ageing research continues to rely on narrow predictors, conservative models, and weak validation. Addressing these gaps requires integrative, diverse, and transparent approaches that link multidimensional predictors with explainable ML.

2.6 Chapter Summary

This chapter reviewed evidence on ageing and chronic disease across three domains, frailty and physical function, mental health, and nutrition and biological ageing. We also examined how machine learning has been applied to predict later life outcomes. The findings showed that while frailty is partly reversible, it

remains strongly tied to mortality; that mental health both shapes and is shaped by physical decline; and that nutrition interacts with biological, metabolic, and social systems in ways that are often underestimated.

While machine learning has improved prediction accuracy in ageing research, challenges remain in integrating diverse predictors and ensuring interpretability and longitudinal validation.

This study builds on previous research by leveraging TILDA alongside the harmonised dataset linking UK and US cohorts, enabling longitudinal prediction of ageing outcomes. The main contribution lies in integrating nutrition, lifestyle, and clinical predictors, applying temporal modelling, and emphasising interpretability, addressing key limitations identified in prior work. These directions motivate the methodological choices outlined in Chapter ??.

References

- [1] Leonardo Bencivenga, Klara Komici, Pierangela Nocella, Fabrizio Vincenzo Grieco, Angela Spezzano, Brunella Puzone, Alessandro Cannavo, Antonio Cittadini, Graziamaria Corbi, Nicola Ferrara, and Giuseppe Rengo. Atrial fibrillation in the elderly: A risk factor beyond stroke. *Ageing Research Reviews*, 61:101092, 2020. ISSN 1568-1637. doi: 10.1016/j.arr.2020.101092. 1
- [2] Laura A Bardon, Melanie Streicher, Clare A Corish, Michelle Clarke, Lauren C Power, Rose Anne Kenny, Deirdre M O'Connor, Eamon Laird, Eibhlis M O'Connor, Marjolein Visser, Dorothee Volkert, Eileen R Gibney, and MaNuEL Consortium. Predictors of Incident Malnutrition in Older Irish Adults from the Irish Longitudinal Study on Ageing Cohort—A MaNuEL study. *J Gerontol A Biol Sci Med Sci*, 75(2):249–256, January 2020. ISSN 1079-5006. doi: 10.1093/gerona/gly225. 1, 11, 14, 15
- [3] James R. C. Davis, Silvin P. Knight, Orna A. Donoghue, Belinda Hernández, Rossella Rizzo, Rose Anne Kenny, and Roman Romero-Ortuno. Comparison of Gait Speed Reserve, Usual Gait Speed, and Maximum Gait Speed of Adults Aged 50+ in Ireland Using Explainable Machine Learning. *Front Netw Physiol*, 1:754477, 2021. ISSN 2674-0109. doi: 10.3389/fnetp.2021.754477. 1, 13, 14, 16
- [4] Patrick V. Moore, Kathleen Bennett, and Charles Normand. Counting the time lived, the time left or illness? Age, proximity to death, morbidity and prescribing expenditures. *Social Science & Medicine*, 184:1–14, 2017. ISSN 0277-9536. doi: 10.1016/j.socscimed.2017.04.038. 1
- [5] Ying Huang, Yuhao Su, Ying Jiang, and Meilan Zhu. Sex differences in the associations between blood pressure and anxiety and depression scores in a middle-aged and elderly population: The Irish Longitudinal Study on Ageing (TILDA). *Jour-*

REFERENCES

- nal of Affective Disorders*, 274:118–125, September 2020. ISSN 0165-0327. doi: 10.1016/j.jad.2020.05.133. 1
- [6] World Health Organization Europe. New data: Noncommunicable diseases cause 1.8 million avoidable deaths and cost US\$ 514 billion every year, reveals new WHO/Europe report. <https://www.who.int/europe/news/item/27-06-2025-new-data-noncommunicable-diseases-cause-1-8-million-avoidable-deaths-and-cost-us-514-billion-USD-every-year-reveals-new-who-europe-report>, 2025. 1
- [7] Niamh Rice and Charles Normand. The cost associated with disease-related malnutrition in Ireland. *Public Health Nutr*, 15(10):1966–1972, October 2012. ISSN 1475-2727. doi: 10.1017/S1368980011003624. 1
- [8] Caoileann H. Murphy, Eoin Duggan, James Davis, Aisling M. O’Halloran, Silvin P. Knight, Rose Anne Kenny, Sinead N. McCarthy, and Roman Romero-Ortuno. Plasma lutein and zeaxanthin concentrations associated with musculoskeletal health and incident frailty in The Irish Longitudinal Study on Ageing (TILDA). *Experimental Gerontology*, 171:112013, 2023. ISSN 0531-5565. doi: 10.1016/j.exger.2022.112013. 1, 3, 10, 14, 15, 16
- [9] Eamon Laird, Charlotte Lund Rasmussen, Rose Anne Kenny, and Matthew P. Herring. Physical Activity Dose and Depression in a Cohort of Older Adults in The Irish Longitudinal Study on Ageing. *JAMA Netw Open*, 6(7):e2322489, July 2023. ISSN 2574-3805. doi: 10.1001/jamanetworkopen.2023.22489. 1, 9
- [10] C. P. McDowell, R. K. Dishman, M. Hallgren, C. MacDonncha, and M. P. Herring. Associations of physical activity and depression: Results from the Irish Longitudinal Study on Ageing. *Experimental Gerontology*, 112:68–75, 2018. ISSN 0531-5565. doi: 10.1016/j.exger.2018.09.004. 1
- [11] Cathal McCrory, Ciaran Finucane, Celia O’Hare, John Frewen, Hugh Nolan, Richard Layte, Patricia M. Kearney, and Rose Anne Kenny. Social Disadvantage and Social Isolation Are Associated With a Higher Resting Heart Rate: Evidence From The Irish Longitudinal Study on Ageing. *J Gerontol B Psychol Sci Soc Sci*, 71(3):463–473, May 2016. ISSN 1079-5014. doi: 10.1093/geronb/gbu163. 2
- [12] Eamon Laird, Matthew P. Herring, Brian P. Carson, Catherine B. Woods, Cathal Walsh, Rose Anne Kenny, and Charlotte Lund Rasmussen. Physical activity for

REFERENCES

- depression among the chronically ill: Results from older diabetics in the Irish longitudinal study on ageing. *Psychiatry Research*, 326:115274, August 2023. ISSN 0165-1781. doi: 10.1016/j.psychres.2023.115274. 3, 14
- [13] Sebastian Moguilner, Silvin P. Knight, James R. C. Davis, Aisling M. O’Halloran, Rose Anne Kenny, and Roman Romero-Ortuno. The Importance of Age in the Prediction of Mortality by a Frailty Index: A Machine Learning Approach in the Irish Longitudinal Study on Ageing. *Geriatrics (Basel)*, 6(3):84, August 2021. ISSN 2308-3417. doi: 10.3390/geriatrics6030084. 3
- [14] Orna A Donoghue, Belinda Hernandez, Matthew D L O’Connell, and Rose Anne Kenny. Using Conditional Inference Forests to Examine Predictive Ability for Future Falls and Syncope in Older Adults: Results from The Irish Longitudinal Study on Ageing. *J Gerontol A Biol Sci Med Sci*, 78(4):673–682, April 2023. ISSN 1758-535X. doi: 10.1093/gerona/glac156. 3, 14
- [15] Roman Romero-Ortuno, Peter Hartley, James Davis, Silvin P. Knight, Rossella Rizzo, Belinda Hernández, Rose Anne Kenny, and Aisling M. O’Halloran. Transitions in frailty phenotype states and components over 8 years: Evidence from The Irish Longitudinal Study on Ageing. *Archives of Gerontology and Geriatrics*, 95: 104401, 2021. ISSN 0167-4943. doi: 10.1016/j.archger.2021.104401. 8, 14, 16
- [16] Kevin McCarthy, Eamon Laird, Aisling M. O’Halloran, Padraic Fallon, Román Romero Ortuño, and Rose Anne Kenny. Association between metabolic syndrome and risk of both prevalent and incident frailty in older adults: Findings from The Irish Longitudinal Study on Ageing (TILDA). *Experimental Gerontology*, 172:112056, 2023. ISSN 0531-5565. doi: 10.1016/j.exger.2022.112056. 8
- [17] Chuanying Huang, Shuqin Sun, Xiaocao Tian, Tong Wang, Tianyu Wang, Haiping Duan, and Yili Wu. Age modify the associations of obesity, physical activity, vision and grip strength with functional mobility in Irish aged 50 and older. *Archives of Gerontology and Geriatrics*, 84:103895, 2019. ISSN 0167-4943. doi: 10.1016/j.archger.2019.05.020. 8, 14
- [18] Ziggi Ivan Santini, Ai Koyanagi, Stefanos Tyrovolas, Josep Maria Haro, Robert J. Donovan, Line Nielsen, and Vibeke Koushede. The protective properties of Act-Belong-Commit indicators against incident depression, anxiety, and cognitive impairment among older Irish adults: Findings from a prospective community-

REFERENCES

- based study. *Experimental Gerontology*, 91:79–87, 2017. ISSN 0531-5565. doi: 10.1016/j.exger.2017.02.074. 9
- [19] A. L. Juola, M. P. Bjorkman, H. Kautiainen, S. Pylkkanen, U. H. Finne-Soveri, H. Soini, K. H. Pitkälä, and J. S. Bell. O1.17: Nursing staff education to reduce potentially harmful medication use among older people in assisted living facilities: Effects of randomized controlled trial on cognition and falls. *European Geriatric Medicine*, 5:S50–S51, 2014. ISSN 1878-7649. doi: 10.1016/S1878-7649(14)70100-7. 9
- [20] Louis Jacob, Karel Kostev, Jae Il Shin, Lee Smith, Hans Oh, Adel S. Abduljabbar, Josep Maria Haro, and Ai Koyanagi. Falls increase the risk for incident anxiety and depressive symptoms among adults aged ≥ 50 years: An analysis of the Irish longitudinal study on ageing. *Archives of Gerontology and Geriatrics*, 114:105098, 2023. ISSN 0167-4943. doi: 10.1016/j.archger.2023.105098. 9, 14
- [21] Kevin McCarthy, Eamon Laird, Aisling M. O’Halloran, Cathal Walsh, Martin Healy, Annette L. Fitzpatrick, James B. Walsh, Belinda Hernández, Padraic Fallon, Anne M. Molloy, and Rose Anne Kenny. Association between vitamin D deficiency and the risk of prevalent type 2 diabetes and incident prediabetes: A prospective cohort study using data from The Irish Longitudinal Study on Ageing (TILDA). *EClinicalMedicine*, 53:101654, November 2022. ISSN 2589-5370. doi: 10.1016/j.eclinm.2022.101654. 10
- [22] Kevin McCarthy, Aisling M. O’Halloran, Padraic Fallon, Rose Anne Kenny, and Cathal McCrory. Metabolic syndrome accelerates epigenetic ageing in older adults: Findings from The Irish Longitudinal Study on Ageing (TILDA). *Experimental Gerontology*, 183:112314, 2023. ISSN 0531-5565. doi: 10.1016/j.exger.2023.112314. 11
- [23] Zahra Azizi, Rebecca J. Hirst, Alan O’ Dowd, Cathal McCrory, Rose Anne Kenny, Fiona N. Newell, and Annalisa Setti. Evidence for an association between allostatic load and multisensory integration in middle-aged and older adults. *Archives of Gerontology and Geriatrics*, 116:105155, 2024. ISSN 0167-4943. doi: 10.1016/j.archger.2023.105155. 11
- [24] Xuezhong Xu, Xue Mingyang, Jie Yang, Hailong Zheng, and Zhifei Che. Tailored machine learning for evaluating the long-term diabetes risk in older individuals: Findings from the Irish Longitudinal Study on Ageing (TILDA). *BMJ Open*, 13

REFERENCES

- (5):e072991, May 2023. ISSN 2044-6055. doi: 10.1136/bmjopen-2023-072991. 11, 12, 13, 14, 15, 16
- [25] Rory Boyle, Lee Jollans, Laura M. Rueda-Delgado, Rossella Rizzo, Görsev G. Yener, Jason P. McMorrow, Silvin P. Knight, Daniel Carey, Ian H. Robertson, Derya D. Emek-Savaş, Yaakov Stern, Rose Anne Kenny, and Robert Whelan. Brain-predicted age difference score is related to specific cognitive functions: A multi-site replication analysis. *Brain Imaging Behav*, 15(1):327–345, February 2021. ISSN 1931-7565. doi: 10.1007/s11682-020-00260-3. 11, 14
- [26] A. A. Zarudsky and K. I. Prashchayeu. P277: One-legged balance test and falls in old patients with cardiovascular diseases. *European Geriatric Medicine*, 5:S171–S172, 2014. ISSN 1878-7649. doi: 10.1016/S1878-7649(14)70448-6. 12, 13, 14
- [27] Soraya Matthews, Mark Ward, Anne Nolan, Charles Normand, Rose Anne Kenny, and Peter May. Predicting mortality in The Irish Longitudinal Study on Ageing (TILDA): Development of a four-year index and comparison with international measures. *BMC Geriatr*, 22:510, June 2022. ISSN 1471-2318. doi: 10.1186/s12877-022-03196-z. 12, 15, 16
- [28] James Davis, Silvin P. Knight, Rossella Rizzo, Orna A. Donoghue, Rose Anne Kenny, and Roman Romero-Ortuno. A linear regression-based machine learning pipeline for the discovery of clinically relevant correlates of gait speed reserve from multiple physiological systems. In *2021 29th European Signal Processing Conference (EUSIPCO)*, pages 1266–1270, August 2021. doi: 10.23919/EUSIPCO54536.2021.9616187. 13, 16
- [29] Zuo Zhang, Lauren Robinson, Robert Whelan, Lee Jollans, Zijian Wang, Frauke Nees, Congying Chu, Marina Bobou, Dongping Du, Ilinca Cristea, Tobias Banaschewski, Gareth J. Barker, Arun L. W. Bokde, Antoine Grigis, Hugh Garavan, Andreas Heinz, Rüdiger Brühl, Jean-Luc Martinot, Marie-Laure Paillère Martinot, Eric Artiges, Dimitri Papadopoulos Orfanos, Luise Poustka, Sarah Hohmann, Sabina Millenet, Juliane H. Fröhner, Michael N. Smolka, Nilakshi Vaidya, Henrik Walter, Jeanne Winterer, M. John Broulidakis, Betteke Maria van Noort, Argyris Stringaris, Jani Penttilä, Yvonne Grimmer, Corinna Insensee, Andreas Becker, Yuning Zhang, Sinead King, Julia Sinclair, Gunter Schumann, Ulrike Schmidt, and Sylvane Desrivières. Machine learning models for diagnosis and risk prediction

REFERENCES

in eating disorders, depression, and alcohol use disorder. *Journal of Affective Disorders*, 379:889–899, June 2025. ISSN 0165-0327. doi: 10.1016/j.jad.2024.12.053.

14